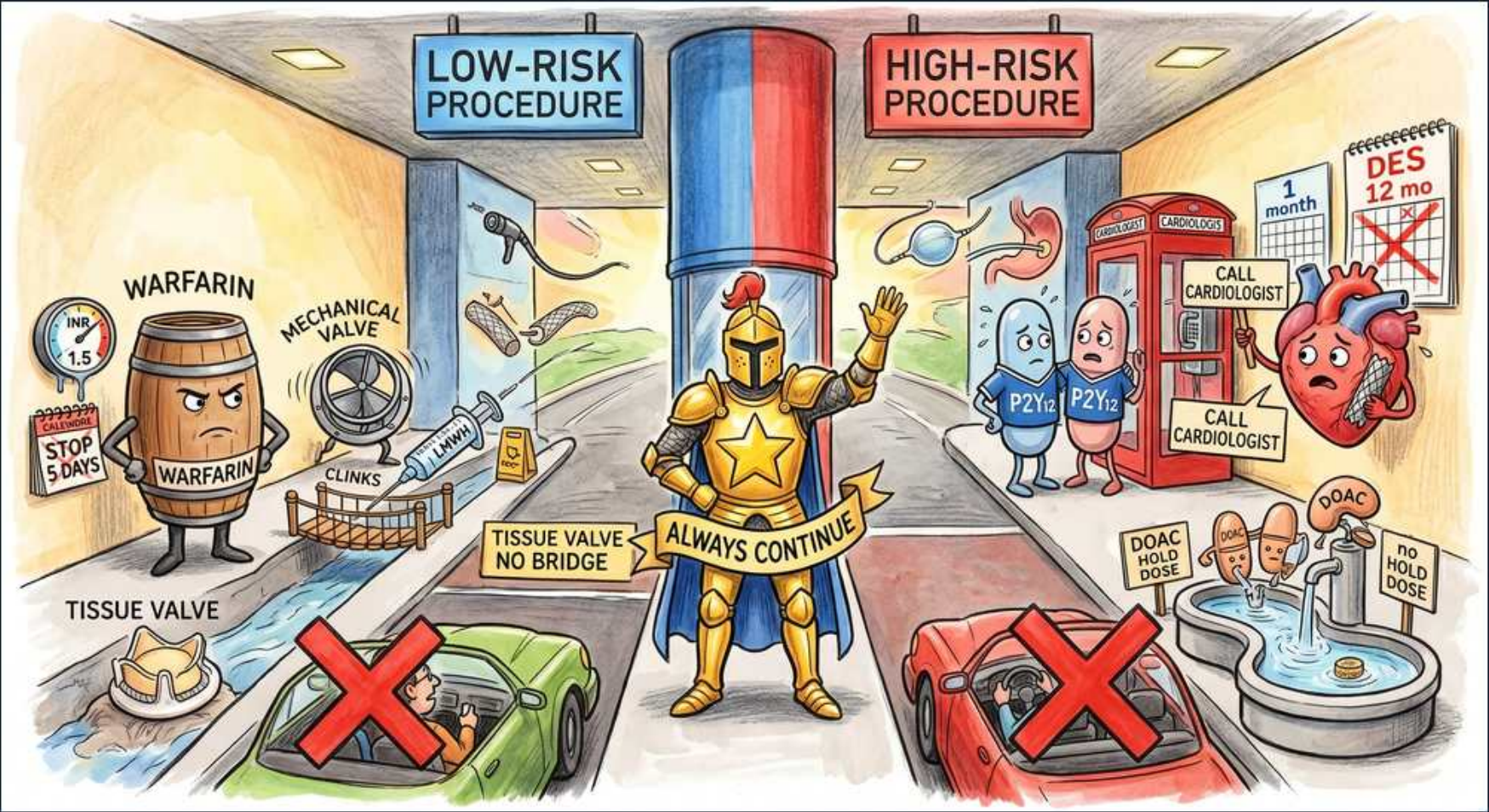


Endoscopy — Antithrombotic Management



1. LOW-RISK procedure sign

WHAT YOU SEE

Blue 'LOW-RISK PROCEDURE' banner over the left half of the hall.

WHAT IT ENCODES

Low-risk (diagnostic ± biopsy, ERCP without sphincterotomy, stent without dilation) → CONTINUE anticoagulation. Bleeding risk is minimal so the thrombotic risk of stopping outweighs it. Decide hold/continue by procedure bleeding risk first, then by patient thrombotic risk.

2. HIGH-RISK procedure sign

WHAT YOU SEE

Red 'HIGH-RISK PROCEDURE' banner over the right half.

WHAT IT ENCODES

High-bleeding-risk procedures = polypectomy, sphincterotomy, EUS-FNA, dilation, ampullectomy, PEG, variceal therapy → HOLD anticoagulation. Aspirin is the exception and is still continued even here.

3. Aspirin knight — ALWAYS CONTINUE

WHAT YOU SEE

Golden knight with a gold star holding an 'ALWAYS CONTINUE' banner, center.

WHAT IT ENCODES

GOLD STAR: aspirin is continued through virtually all endoscopy, including high-risk procedures, in patients with established cardiovascular/cerebrovascular disease. Stopping it raises thrombotic events without a meaningful drop in clinically significant GI bleeding. Only consider holding for the very highest-risk resections, and even then case-by-case.

BOARD TRAP

'Stop aspirin before any polypectomy' → wrong; aspirin is continued — stopping increases cardiovascular/thrombotic events.

4. Warfarin barrel — STOP 5 DAYS

WHAT YOU SEE

Angry brown barrel labeled WARFARIN with an INR gauge and 'STOP 5 DAYS' tag, upper left.

WHAT IT ENCODES

For high-risk procedures stop warfarin ~5 days before; confirm INR ≤1.5 on the day. Resume the evening of the procedure once hemostasis is secure. Whether to bridge depends entirely on thrombotic risk, not on the warfarin itself.

5. Mechanical valve — BRIDGE

WHAT YOU SEE

Mechanical valve gear labeled 'CLINKS' with an LMWH syringe, upper left.

WHAT IT ENCODES

HIGH thrombotic risk → bridge with LMWH (or UFH) while INR is sub-therapeutic. High-risk = mechanical mitral valve, any caged-ball/older mechanical valve, AF with prior stroke, VTE within 3 months. Bridge LMWH stopped ~24h pre-procedure.

6. Tissue valve — NO BRIDGE (red X)

WHAT YOU SEE

Gold tissue/bioprosthetic valve lower left with a green car and a red X.

WHAT IT ENCODES

A tissue/bioprosthetic valve is LOW thrombotic risk and does NOT need bridging — just hold and resume warfarin. Bridging here only adds bleeding risk with no thrombotic benefit. Same 'no bridge' logic applies to non-valvular AF without prior stroke.

BOARD TRAP

'Bridge a bioprosthetic valve patient with LMWH' → wrong; tissue valves are low-risk — no bridging, bridging only causes bleeding.

7. P2Y12 (clopidogrel) pills

WHAT YOU SEE

Two blue 'P2Y12' capsule characters by the red phone booth, right of center.

WHAT IT ENCODES

P2Y12 inhibitors (clopidogrel, prasugrel, ticagrelor) for high-bleeding procedures may be held ~5–7 days — but ONLY if the coronary-stent window allows and after cardiology input. Aspirin is continued throughout when on dual therapy.

8. Call cardiology phone booth

WHAT YOU SEE

Red phone booth labeled 'CALL CARDIOLOGIST', right side.

WHAT IT ENCODES

Never stop a P2Y12 inhibitor in the early post-stent window without consulting cardiology; defer elective endoscopy until the window passes. Premature interruption → stent thrombosis, which carries high mortality.

9. Bare-metal stent — 1 month

WHAT YOU SEE

Calendar marked '1 month' upper right.

WHAT IT ENCODES

Bare-metal stent (BMS): minimum ~1 month of dual antiplatelet therapy before P2Y12 can be safely interrupted for elective endoscopy. Delay elective high-risk procedures until then.

10. Drug-eluting stent — 12 mo (red X)

WHAT YOU SEE

Calendar marked 'DES 12 mo' with a red X, far upper right; an anxious heart below.

WHAT IT ENCODES

Drug-eluting stent (DES): continue dual antiplatelet ~6–12 months; do NOT stop P2Y₁₂ within this window for an elective procedure. If endoscopy is truly urgent within the window, manage with cardiology and continue aspirin.

BOARD TRAP

'Stop clopidogrel for elective polypectomy 2 months after a drug-eluting stent' → wrong; within the ~12-month window stopping risks fatal stent thrombosis — delay or consult cardiology.

11. DOAC — HOLD, no bridge

WHAT YOU SEE

Pills at a fountain labeled 'DOAC HOLD DOSE' / 'no hold dose', lower right.

WHAT IT ENCODES

DOACs (apixaban, rivaroxaban, dabigatran, edoxaban) → HOLD 1–2 days before high-risk procedures, longer if renal impairment (especially dabigatran with low CrCl). Their short half-life means NO bridging is needed. Resume ~24–48h post-procedure once hemostasis is confirmed.

BOARD TRAP

'Bridge a DOAC patient with LMWH around endoscopy' → wrong; DOACs have short half-lives — just hold and resume, never bridge.